From: Nicholas Scarr <nscarr@quantlogic.com>
To: Andy Lyne <Andy.Lyne@qlaims.com>
CC: Claims <Claims@qlaims.com>, Louisa Scarr <scarrlouisa@gmail.com>
Date: Monday 03 November 2025 at 17:57
Subject: 1 Riverside Cottages – Confirmation of Professional Understanding Following Meeting

Hi Andy,

Please see below the email I am suggesting you send. Darren definitely said he was surprised the kitchen units had not already been BER'd but I cannot be certain whether you were on the phone when he said that. Do you recall? If not we can amend the email slightly reflecting the comment was made to me.

Trust this otherwise represents a clear and factual account of our discussion. Here is the text:-

From: Andy Lyne andy.lyne@qlaims.com
To: Darren [Surname] darren@bioresponse.co.uk
Date: [Insert Date]

Dear Darren,

Further to our meeting at BioResponse's offices last Thursday, attended in person by *Mr Nicholas Scarr* and by telephone by myself, I wanted to record our shared understanding of the position at *1 Riverside Cottages* and to confirm the factual and professional points discussed.

Please review the summary below and let me know if it aligns with BioResponse's professional position. If any point is inaccurate, I would be grateful if you could set out the technical basis for your view.
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1. Chronology and Extent of Contamination

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A sewage ingress occurred on *28 February 2025*, affecting both the *interior* of the property and the *front and rear external areas*.
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The property remained contaminated for a prolonged period while investigations and drying arrangements were organised.
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*Most household contents and building materials were present in this

environment for approximately three months*, with *some remaining for up to five months*, until the final strip-out was completed towards the end of July 2025.

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Throughout this time the dwelling was subject to *persistent dampness and high humidity*, consistent with *Category 3 (sewage)* contamination.

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These conditions would have enabled *airborne migration of bacteria, fungal spores and other pathogens* throughout the building, depositing on and within porous materials across both floors.

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2. Medical Backdrop (recorded for context, not for agreement)

It is relevant to record the professional opinion of *Dr Vanya Gant*, Consultant Physician in Cellular Immunology and Infectious Diseases, who has been directly involved in Mrs Louisa Scarr's care. Dr Gant confirms that:

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Mrs Scarr is *clinically vulnerable*, having undergone major surgery, radiotherapy for breast cancer, and now living with *advanced chronic kidney disease* while carrying a *multidrug-resistant Gram-negative organism*.

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This combination places her at a *significantly heightened risk of both bacterial and fungal infection*, the latter occurring through inhalation of airborne mould spores which may also trigger hypersensitivity reactions.

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Dr Gant describes sewage as *"one of the richest sources of both bacteria and moulds known to man,"* noting that residual dampness will hugely increase airborne mould levels and that it is therefore *"very important for the preservation of Mrs Scarr's future health to make every effort to reverse the totality of the sewage-related damage … including double deep cleaning to hospital standards."*

This medical evidence establishes the level of cleanliness required to render the property safe. In technical terms, the same reasoning applies across all *porous or absorbent materials*—plasterboard, joinery, insulation, timbers, kitchen units and furnishings—each of which can retain or re-emit airborne microbial contamination under damp conditions.

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3. Airborne Contamination and Implications for Materials

As discussed, the extended period during which the property and its contents remained in a sewage-contaminated, high-humidity environment would have led to *widespread airborne dissemination and deposition* of microbial material.

Porous substrates—particularly kitchen units, timber joinery, plaster surfaces and insulation—are prone to absorbing and trapping these contaminants.

Given the nature and duration of the exposure, such materials *cannot be decontaminated to a standard consistent with hospital-grade sterility* and must therefore be *removed and replaced*.

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4. BioResponse's Professional Position (as confirmed in our meeting)

You confirmed that:

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There is *no economically viable method* to verify the complete elimination of pathogens from porous or absorbent materials once exposed to sewage;

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Surface or ATP swab testing is limited to the exact point and time tested and *cannot evidence systemic sterility*; and

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Any attempt to restore such materials to hospital standards would be *uneconomic and scientifically unreliable*.

During our discussion you also remarked that you were *surprised the kitchen units had not already been classified as Beyond Economic Repair (BER)*, given their proximity to contaminated areas and prolonged exposure.

Accordingly, the required scope includes:

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Removal and replacement of *porous and absorbent materials*, including *kitchen units*, joinery, plasterboard/plaster, insulation, timbers and soft furnishings;

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Renewal of *electrical systems and vented fittings* exposed to contaminated air, in line with *NICEIC / BS 7671*; and

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Full internal redecoration throughout, reflecting the prolonged airborne exposure and humidity.

I would be grateful if you could confirm in writing that the above accurately reflects BioResponse's professional understanding and our discussion.
Please provide your response *at your earliest convenience* so that this record may be finalised.

With thanks for your cooperation and professional input.

Kind regards,
Andy Lyne
Independent Loss Adjuster
Qlaims
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